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Santa Cruz County Board of Supervisors buck staff advice, preserve in-house radiology services

Jimmy Dale, Santa Cruz County Finance Director, speaks with the Santa Cruz County Board of Supervisors Tuesday morning, Sept. 9, during a special session in Santa Cruz on Wednesday. The board ultimately voted to keep in-house radiology services for one year after transferring operations, offering to pay a consultant to help with the transition. (Shmuel Thaler — Santa Cruz Sentinel)

By Tim Cornell, tcornell@santacruzsentinel.com | Santa Cruz Sentinel
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SANTA CRUZ — In a partial rejection of the recommendations given by its staff, the Santa Cruz County Board of Supervisors decided to preserve some primary care services offered at its health clinics that were previously declared for the chopping block, while following through with plans to shutter its in-house radiology services.

At its county clinics, Ventura, the board chose to preserve the in-house radiology services it offers out of its federally qualified health centers in Santa Cruz and Watsonville, along with the two full-time staff positions necessary to run the services for one fiscal year, 2025 to 2026. But in the same motion, with the one dissenting vote from Supervisor Manu Koenig, the local lawmakers instructed county staff to begin transitioning the services to a private radiology contractor or department as part of an effort to bring in a third-party provider — Quest Diagnostics — to take over operations.

Supervisor Kim De Serpa, a former social worker, explained that in her view it would be very challenging to get patients — some of whom struggle with behavioral health or mental illness — to travel to an offsite imaging location, even if travel options were made available.

"I do think being able to truly this population in the clinics is really important," said De Serpa.

After strong protest from Dale Ventura, the county's largest union, the board decided to pick up the spend $300,000 to extend the in-house laboratory services until Sept. 30 before transitioning to a private vendor. Ventura's union represents the staff who performed the services for patients.

Additionally, it dedicated $150,000 to continue with the radiology services, but asked that Health Services Agency staff return Sept. 9 with alternative recommendations on the 12 active county employees that would be impacted by the reductions. Of that total, six employees were placed in another county department, two retired, one resigned, one is still working pro-rata and one will be laid off Sept. 30.

The radiology crisis within the system made an about $500,000, while the laboratory services are about $1.2 million. To fund both services another nine months would have cost the county another $1.65 million, according to the staff report.

But after pining over the items in the budget once more, Health Services Agency leaders found a way to preserve the services for a transition period, while still following through on its proposal. At Tuesday's meeting, Health Services Agency Assistant Director Jennifer Herrera explained that expenses for these and other services offered at the county health centers — including primary care services, which the board decided to keep in its budget last June — have been trending down since the fiscal year 2023 to 2024. But in the same time period, the county has seen its revenues from the county's budget that was adopted in June and is covered to be formally finalized Sept. 30.

County Health Services Agency Chief of Health Centers Amy Pender added that there are all kinds of reasons why patients are coming to the county's clinics less frequently, including California — that are served locally, but the county is trying to adapt in the tough trend it has been dealt.

"We detour lot of our different services — lab and radiology stand out. They are essential services for primary care, but there are options to provide these services to patients in a community-based environment," said Pender. "We are seeing patients coming to the clinic greatly reduced compared to the fiscal impact on the number of staff hours spent, at the same time, they have great value to our patients."

Part of the board's direction is prior was for county staff to collaborate with 30's leadership as they develop alternatives for the lab and radiology services. The union has continued to express concerns about patient safety stations and creative revenue generation strategies could help close the funding gap.

But Supervisor Justin Cummings, in comments that were partially echoed by Supervisor Monica Martinez and Chair Felipe Hernandez, expressed frustration that not enough options were shared by county staff at Tuesday's meeting. He asked for better collaboration between the county and 30's and suggested the Health Services Agency hire a Director of Strategic Initiatives, Grants and Revenue Maximization, who would work a four-year job offer.

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"I get it. We are seeing more and more single-time, but it also seems to remind the board that we do have a new Health Services Agency director, it's only working, I think there's an opportunity for us to potentially have her work with frontline workers and really try to address some of these issues."

She (Director Laura, Santa Cruz chapter president for 50's 52), told the Sentinel after the meeting that she hopes that board remind a "new set of expectations."

"We understand the harsh economic reality that we're all moving towards, but I commend the board for open demonstrating that management work with us and come up with solutions rather than making unilateral decisions on their own and leaving us out in the dark."

Prior to the board's vote, Health Services Agency Director Elizabeth Lee gave a presentation in conversation with a crew of sobering question for the county leaders to weigh as they made their decision.

Palacios recognized that steps to ramp up revenue might be viable in the future, but don't do much to address the immediate budget shortfall. Lee said the Health Services Agency will look at an entire fiscal year to plan the radiology services, but it's on a gradual timeline to ramp up to use one-time funding for ongoing operations.

If the board must depart from the staff recommendation, he concluded, the best solution would be for the board to take another look at its general fund and draw revenue away from other departments to preserve services. Palacios said he hopes the board and the public remember that many those difficult budgetary decisions are not the one-and-done forever.

"This is just the beginning of cuts that are coming to Health Services Agency," he said.

The county has already experienced an estimated 5% drop in Medi-Cal coverage in recent months, Palacios explained, and as cuts from the federal bill continue to roll out, health care experts are expecting 20% to 25% of Medi-Cal beneficiaries will lose coverage. This will spike a particular cycle of demonstrated services and less revenue from federal reimbursements.

Lee said the Health Services Agency will be ready to work with the board and staff on ways to offset the forgoing and ending funds in a better pace to ease impact. Unfortunately, "We don't have the luxury of a warm fire. But the Congress passed (the bill) the president signed it — it's his initiative — that's what they're doing."

Koenig lamented the decision and once again to exploring long-term changes to shore up the county's finances, but felt the board needed to see the decision from pure all the way through.

"We have dangerously low reserves right now, I mean, historically low reserves," said Koenig, "and I think our staff wants to be making these recommendations. This is just the hard option."

The motion directed county staff to return at the Sept. 30 meeting with a plan to fund the radiology services for that year (2025 to 2026 using one-time funds. It also asked the Health Services Agency director to explore strategies that address revenue and productivity issues, including the feasibility of a third-party provider, and to return to the board with a report on staff as well with 30's on strategies for maintaining radiology and county health care services more broadly.

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